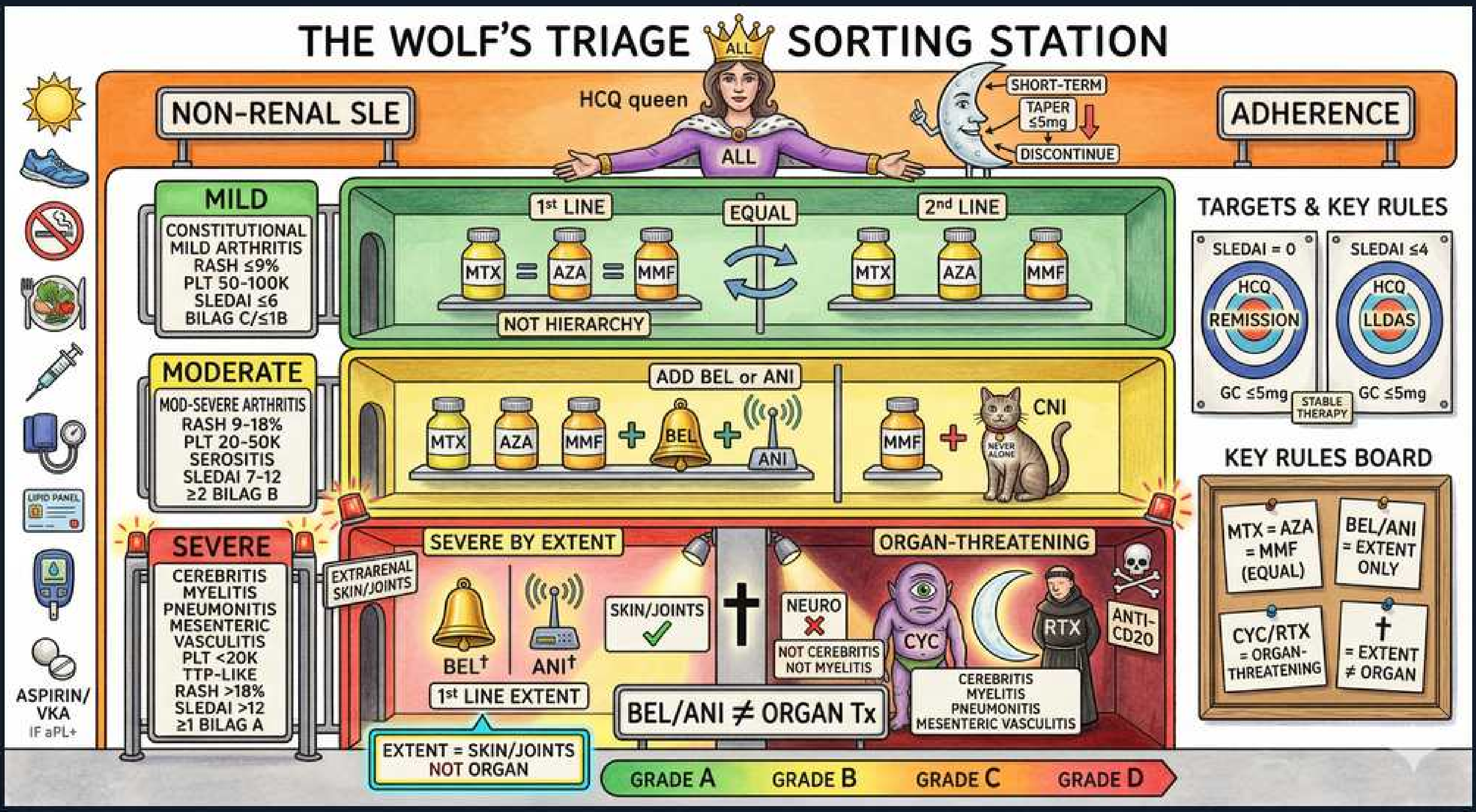
SLE — Non-Renal Management & Escalation



1. HCQ backbone for ALL

WHAT YOU SEE

A crowned HCQ 'queen' on a throne over a banner reading ALL — she rules every SLE patient regardless of severity, the one drug everyone stays on.

WHAT IT ENCODES

Hydroxychloroquine is the foundational therapy for EVERY SLE patient regardless of severity (the 'queen' ruling over all). It reduces flares, prevents organ damage, lowers thrombosis risk, improves lipids/glucose, and independently improves SURVIVAL. It is also safe and recommended to continue throughout pregnancy. Typical dosing is ≤5 mg/kg actual body weight (often ~200–400 mg/day) to limit retinal toxicity.

BOARD TRAP

WRONG: stopping hydroxychloroquine to 'calm down' a flare or during pregnancy. Discriminator: HCQ should be CONTINUED through flares and pregnancy because withdrawal precipitates flares and worsens outcomes — it is the one drug essentially all lupus patients stay on long-term.

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2. HCQ retinopathy monitoring

WHAT YOU SEE

The targets board HCQ REMISSION / LLDAS with an eye note — the eye flags the dose-limiting bull's-eye retinopathy that mandates baseline + annual OCT screening.

WHAT IT ENCODES

The dose-limiting toxicity of hydroxychloroquine is irreversible RETINOPATHY (bull's-eye maculopathy). Keep the dose ≤5 mg/kg/day and obtain a BASELINE ophthalmologic exam within the first year, then ANNUAL screening after 5 years (sooner with risk factors: high dose, renal impairment, tamoxifen use, pre-existing retinal disease). Screening uses automated visual fields plus spectral-domain OCT.

BOARD TRAP

WRONG: waiting for visual symptoms before screening, or relying on a simple eye-chart check. Discriminator: HCQ retinopathy is detected by OCT and automated visual fields BEFORE symptoms appear and is irreversible once symptomatic — so structured baseline-plus-annual objective screening is required, not symptom-driven evaluation.

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3. Minimize glucocorticoids

WHAT YOU SEE

A moon labeled SHORT-TERM TAPER / DISCONTINUE — the Cushingoid 'moon face' reminds you steroids are only a short bridge to be tapered off, not maintenance.

WHAT IT ENCODES

Glucocorticoids control disease quickly but cause most of the long-term DAMAGE in lupus (osteoporosis, avascular necrosis, infection, diabetes, cataracts, hypertension). Use them as a SHORT-TERM bridge while steroid-sparing agents take effect, then taper to ≤5 mg/day prednisone or off entirely. The moon (Cushingoid moon face) is the reminder of steroid harm; pulse IV methylprednisolone is reserved for severe organ-threatening flares.

BOARD TRAP

WRONG: maintaining chronic moderate/high-dose prednisone for ongoing control. Discriminator: steroids are a bridge, not maintenance — chronic high-dose use drives irreversible SLICC damage; the strategy is to add a steroid-sparing immunosuppressant early and taper steroids to ≤5 mg/day (ideally off).

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4. Mild SLE

WHAT YOU SEE

A green MILD panel listing rash <9%, arthritis, platelets >50-100k — green = treat conservatively (HCQ ± brief low-dose steroids).

WHAT IT ENCODES

Mild SLE (low SLEDAI) = constitutional symptoms, mild non-deforming arthritis, limited cutaneous disease (rash <9% BSA), and mild thrombocytopenia (platelets ~50–100k) — no major organ threat. Management: hydroxychloroquine plus short low-dose steroids for flares, and NSAIDs for arthralgia/serositis. The green panel signals 'safe to treat conservatively.'

BOARD TRAP

WRONG: starting cyclophosphamide or rituximab for mild disease. Discriminator: mild SLE is managed with HCQ ± brief low-dose steroids (and a conventional immunosuppressant if HCQ is insufficient) — reserve cytotoxic/biologic escalation for moderate-to-severe or organ-threatening disease.

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5. Mild 1st line: MTX/AZA/MMF

WHAT YOU SEE

Three equal bottles MTX, AZA, MMF under the word EQUAL with a '≠ hierarchy' note — they are interchangeable first-line steroid-sparers chosen by patient profile, not a fixed ladder.

WHAT IT ENCODES

When HCQ alone is inadequate, conventional steroid-sparing immunosuppressants — methotrexate, azathioprine, or mycophenolate (MMF) — are added, chosen by PATIENT PROFILE rather than a fixed hierarchy (they are roughly 'EQUAL' options). Key selection drivers: AZA is pregnancy-safe; MTX is good for arthritis but teratogenic and needs folate; MMF favors renal/severe skin involvement but is teratogenic.

BOARD TRAP

WRONG: treating these as a mandatory sequential ladder, or giving MTX/MMF to a patient planning pregnancy. Discriminator: MTX, AZA, and MMF are comparable first-line steroid-sparers selected by comorbidity and pregnancy status — AZA is the pregnancy-compatible choice, whereas MTX and MMF are contraindicated in pregnancy.

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6. Moderate SLE

WHAT YOU SEE

A yellow MODERATE panel (rash 9-18%, serositis, platelets 20-50k) — yellow = step up to a steroid-sparing agent, add a biologic if refractory.

WHAT IT ENCODES

Moderate SLE (SLEDAI ~7–12) = more significant arthritis, more extensive rash (9–18% BSA), moderate thrombocytopenia (platelets ~20–50k), and serositis (pleuritis/pericarditis) — beyond mild but not yet organ-threatening. Management adds a conventional immunosuppressant and, if control is inadequate, a biologic (belimumab or anifrolumab) while minimizing steroids.

BOARD TRAP

WRONG: under-treating moderate disease with HCQ alone, or jumping straight to cyclophosphamide. Discriminator: moderate SLE warrants a steroid-sparing immunosuppressant and, if refractory, add-on belimumab/anifrolumab — cyclophosphamide and rituximab are reserved for severe, organ-threatening disease.

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7. Add belimumab / anifrolumab

WHAT YOU SEE

A signpost reading ADD BEL or ANI — these biologics are layered ON TOP of HCQ + standard therapy (add-on, never the backbone).

WHAT IT ENCODES

For inadequately controlled disease, ADD a targeted biologic to standard therapy: belimumab (a monoclonal antibody against BLyS/BAFF that depletes autoreactive B cells) or anifrolumab (a monoclonal antibody blocking the type I interferon receptor). Both are particularly useful for refractory skin and joint disease and have steroid-sparing effects; belimumab also has evidence in lupus nephritis as add-on.

BOARD TRAP

WRONG: using belimumab or anifrolumab as stand-alone monotherapy or as a substitute for hydroxychloroquine. Discriminator: these biologics are ADD-ON agents layered onto HCQ + standard immunosuppression in patients who remain active despite first-line therapy — not replacements for the backbone.

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8. CNI option

WHAT YOU SEE

A cat labeled CNI — the calcineurin-inhibitor add-on (tacrolimus/voclosporin), especially valuable for membranous/proteinuric renal disease.

WHAT IT ENCODES

Calcineurin inhibitors — tacrolimus and voclosporin — can be added, especially with renal involvement; voclosporin combined with MMF is approved for active lupus nephritis and is favored with membranous (class V) or mixed-class disease and nephrotic-range proteinuria. The 'CNI cat' is the visual hook. They reduce proteinuria via both immunosuppressive and direct podocyte-stabilizing effects.

BOARD TRAP

WRONG: forgetting to monitor for CNI-specific toxicity. Discriminator: calcineurin inhibitors require monitoring of renal function, blood pressure, and drug levels (nephrotoxicity, hypertension, neurotoxicity) — they are add-on agents particularly valuable for membranous/proteinuric lupus nephritis, not generic first-line SLE drugs.

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9. Severe SLE

WHAT YOU SEE

A red SEVERE panel listing cerebritis, myelitis, vasculitis, nephritis, low platelets — red = organ-threatening, demanding pulse steroids + CYC or RTX.

WHAT IT ENCODES

Severe SLE (SLEDAI >12) = MAJOR ORGAN-THREATENING disease: proliferative lupus nephritis, neuropsychiatric lupus (cerebritis, myelitis), pneumonitis/alveolar hemorrhage, systemic vasculitis, and life-threatening cytopenias (severe AIHA, TTP-like TMA). The red panel mandates aggressive immunosuppression — high-dose/pulse steroids PLUS cyclophosphamide or rituximab (and MMF for nephritis).

BOARD TRAP

WRONG: managing organ-threatening lupus with HCQ and low-dose steroids alone. Discriminator: severe/organ-threatening SLE requires INDUCTION with high-dose glucocorticoids plus a potent agent (cyclophosphamide, rituximab, or MMF for nephritis) — failure to escalate risks irreversible organ loss or death.

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10. Severe by extent (skin/joints)

WHAT YOU SEE

A SEVERE BY EXTENT — SKIN/JOINTS, NOT ORGAN box with a ringing BEL/ANI bell — widespread skin/joint disease that escalates to a biologic, not cyclophosphamide.

WHAT IT ENCODES

Some disease is 'severe by EXTENT' — widespread, disabling skin or joint involvement that is NOT organ-threatening. This is escalated differently from organ-threatening disease: optimize HCQ and conventional immunosuppressants, then ADD belimumab or anifrolumab (the bell), rather than reaching for cyclophosphamide. The distinction (extent vs organ threat) drives the escalation path.

BOARD TRAP

WRONG: treating extensive skin/joint disease with cyclophosphamide as if it were organ-threatening. Discriminator: severity by EXTENT (diffuse skin/joints, no vital organ at risk) → biologic add-on (belimumab/anifrolumab); severity by ORGAN THREAT (nephritis, CNS, vasculitis) → cyclophosphamide/rituximab. The escalation choice depends on which kind of severe it is.

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11. Organ-threatening neuro

WHAT YOU SEE

A brain labeled NEURO with a 'NOT cerebritis vs thrombosis' caution — true inflammatory NPSLE gets immunosuppression, but APS stroke gets anticoagulation instead.

WHAT IT ENCODES

Organ-threatening neuropsychiatric SLE (diffuse cerebritis, severe cognitive dysfunction, seizures, psychosis) requires aggressive immunosuppression with high-dose/pulse glucocorticoids plus cyclophosphamide or rituximab. First, exclude mimics — infection, metabolic derangement, drug effects (especially steroid psychosis), and APS-related stroke (which needs ANTICOAGULATION, not immunosuppression).

BOARD TRAP

WRONG: immediately immunosuppressing every neuropsychiatric event in a lupus patient. Discriminator: a thrombotic stroke from antiphospholipid syndrome is treated with ANTICOAGULATION, and steroid psychosis is treated by reducing steroids — only true inflammatory NPSLE warrants escalating immunosuppression, so distinguish the mechanism first.

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12. CYC for organ-threatening

WHAT YOU SEE

A CYC bottle beside cerebritis/myelitis/vasculitis labels — the heavy alkylating agent reserved for organ-threatening disease (needs MESNA + ovarian protection).

WHAT IT ENCODES

Cyclophosphamide is the potent alkylating agent for ORGAN-THREATENING lupus — severe CNS disease (cerebritis, transverse myelitis), systemic vasculitis, pneumonitis, and proliferative nephritis. The low-dose 'Euro-Lupus' regimen (500 mg IV q2 weeks ×6) is preferred for nephritis to limit toxicity. Key adverse effects: gonadal failure/infertility (give GnRH agonist for ovarian protection), hemorrhagic cystitis and bladder cancer (give MESNA + hydration), myelosuppression, and infection.

BOARD TRAP

WRONG: using cyclophosphamide without fertility and bladder protection, or for mild/moderate disease. Discriminator: CYC is reserved for organ-threatening disease and demands MESNA + hydration (to prevent hemorrhagic cystitis/bladder cancer) and ovarian protection (GnRH agonist) — it is not a first-line agent for mild or extent-only severe disease.

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13. Rituximab (anti-CD20)

WHAT YOU SEE

A vial labeled RTX / ANTI-CD20 — B-cell depletion for refractory/severe disease and severe cytopenias (screen hepatitis B first).

WHAT IT ENCODES

Rituximab (anti-CD20 B-cell depletion) is used off-label for REFRACTORY or severe disease, particularly severe autoimmune cytopenias (refractory immune thrombocytopenia, AIHA) and as an alternative in organ-threatening disease intolerant of/failing other agents. Screen and manage for hepatitis B reactivation before infusion; watch for infusion reactions and (rarely) PML.

BOARD TRAP

WRONG: giving rituximab without screening for hepatitis B. Discriminator: B-cell depletion can REACTIVATE hepatitis B (potentially fulminant), so HBV serologies are mandatory before rituximab — it is reserved for refractory/severe disease (notably severe cytopenias), not routine first-line lupus.

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14. GC ≤5mg target, stable

WHAT YOU SEE

A targets sign reading GC ≤5mg / STABLE — the goal is remission/LLDAS on prednisone ≤5 mg/day; needing more means escalate the steroid-sparer.

WHAT IT ENCODES

The overarching treatment TARGETS are remission (DORIS) or, failing that, Lupus Low Disease Activity State (LLDAS), achieved on prednisone ≤5 mg/day with stable disease and no severe flares. Reaching these targets reduces flares, damage accrual, and mortality. Hydroxychloroquine continues throughout, with steroids minimized and steroid-sparing agents used to sustain the target.

BOARD TRAP

WRONG: accepting persistent disease control that relies on chronic prednisone >5 mg/day. Discriminator: the goal is remission/LLDAS with prednisone ≤5 mg/day — if a patient needs more than that to stay quiet, escalate the steroid-sparing regimen rather than maintaining higher steroids, because chronic steroids drive irreversible damage.
